



OFFICIAL

Long Acting Injectable Depot Medication in Community Mental Health Clinics Procedure

1. Scope

Site	Operational Area	Applicable to
AKG	Community Mental Health Clinics	Medical, Nursing and Pharmacy

2. Purpose

The purpose of this procedure is to outline the role and responsibility of clinicians in the safe prescribing, ordering, monitoring and administration of all Long Acting Injectable (LAI) medications in the community mental health teams

Depot injections must only be administered to consumer's that they are prescribed and dispensed for.

This procedure should be read in conjunction with the Armada Kalamunda Group (AKG) [Medication Policy](#) and the AKG [Medication Management Procedure](#).

Olanzapine LAI:

There is a risk of post injection syndrome (PIS) with every dose of Olanzapine (Zyprexa Relprevv[®]) and consumers must be monitored for signs and symptoms of PIS and/or delirium for at least 2 hours after each dose. Consumers must consent to:

Being monitored post injection prior to administration

Not driving or operating machinery for the remainder of the day post administration.

Be able to access emergency medical treatment if signs or symptoms of overdose become apparent.

- For the administration of Olanzapine LAI in the event of PIS occurring, the emergency response for Orchard Avenue Centre (OAC) will be via 000 call for ambulance.
- LAI medication is administered by deep intramuscular injection.
- Olanzapine LAI is administered by gluteal injection only.

3. Background

This service aims to:

- Support the consumer's recovery and independence in attending appointments and accessing treatment.
- Mitigate any risk issues when administration cannot be conducted within the consumer's home.

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Provide flexibility for consumers to attend at a time convenient to them/their support person.
- Administration of antipsychotic depot medication via the community clinics is conducted at OAC Community Mental Health clinic or by domiciliary visits.

4. Prescribing

- Each consumer must have their depot medication prescribed on the Intramuscular Long-Acting Injection Chart (MR58) to allow for administration.
- A written prescription must accompany each chart and be sent to Armadale Hospital Pharmacy to arrange medication supply.
- A new prescription must be written and sent to Armadale Health Service (AHS) Pharmacy along with the updated depot chart if there is a dose or frequency change of the same medication.
- The consumer must be provided with written information on the medication being prescribed..
- Consumers are to be informed of the side effect profile of medication including the risk of PIS with Olanzapine LAI (1.85% of patients were affected in pre-marketing studies – refer to [MIMS Product Information](#)).
- Where possible all prescriptions must meet the relevant PBS prescribing criteria and the State-wide Medicines Formulary (Formulary 1) criteria. Where this is not met a valid Individual Patient Approval (IPA) signed by the Director of Clinical Services is required before the medication can be supplied.
- If required, the prescriber should complete applicable Special Access Scheme (SAS) forms for depots that do not have Australian registered products so that SAS stock is able to be sourced. Prescriber to liaise with AHS Pharmacy before prescribing to assess availability.

5. Training

There is a responsibility on the clinician administering the medication to manage administration within their scope of practice and to raise any concerns with the relevant line manager and/or prescriber.

6. Procedure

6.1 Administration of LAI in Community Clinics

- The Care Coordinator (CC) will take all steps to monitor when a consumer's depot is due, this will include liaison with appropriately qualified staff if they are not familiar with depot medications.
- A Registered Nurse (RN) is responsible for administering the depot as per the AKG Medication Management Policy.
- RN or CC will liaise with relevant prescriber to obtain prescriptions required by AHS Pharmacy. These prescriptions are to be faxed to pharmacy on the same day that the depot order is emailed to ensure timely dispensing and delivery.
- If depot medication is changed, the prescriber must update the medication profile to reflect the change. This includes updating the Intramuscular Long-Acting Injection Chart (MR58) and printing a new prescription. This updated information then needs to be forwarded to Armadale Health Service (AHS) Pharmacy to ensure timely supply.

- Prior to attending to the consumer, the RN will check Psolis, and the depot prescription chart for medication and safety alerts, responding accordingly
- The RN will confirm prior to administration the consumers name, date of birth address and photograph (if consented to photo ID). Photographic ID is to be attached to the consumer's depot medication chart with the consumer's consent. The consumer consent form should be completed by the care coordinator/ RN if the consumer is agreeable to having their photograph taken for ID purposes and stored in the consumer medical record. If the consumer does not give consent, the care coordinator/depot RN must document this in the consumers medical record and on the depot medication chart
- Prior to administration of depot injection, the RN will check with the consumer/in the health care record that they have received information regarding their medication. The RN will provide further information if necessary
- Consent will be gained from the consumer for the medication to be administered in keeping with EMHS policy: Consent and Department of Health Operational Directive OD 0657/16 WA Health Consent to Treatment policy
- For Olanzapine depot the consumer must consent to post monitoring for at least 2 hours. It should be confirmed prior to the planned administration regarding the transportation arrangements for the consumer. Consumers who cannot be conveyed to and from the identified service by a third party will be provided with transport or bus voucher where available due to the driving restriction post administration.
- Prior to administration, the RN will check intended and previous injection site for signs of infection or swelling. In the event of suspected adverse reaction, the RN will take observations (Blood Pressure, Pulse, Temperature, Respirations) and discuss with a doctor prior to administering the depot medication.
- There will be a two person check at point of administration with all depot administration within the clinic. The second person is required to sign and date on the depot chart.
- For community mental health home visits, if a risk assessment permits, an RN conducting a home visit may administer a LAI without a second check at the point of administration, provided a second check is completed and documented before leaving the workplace. The RN may only administer LAI to a single consumer on a single visit. If other LAI administrations are scheduled to be completed outside of the clinic, these must be independently checked before each visit occurs.
- Perform hand hygiene and prepare consumer – use non-sterile gloves as per standard precautions prior to administering the injection. On completion of the procedure remove gloves and perform hand hygiene
- Post administration of depot injection the RN will document on depot chart the following: Date given, site depot administered to, attach sticker or write batch number and expiry date of the medication provided, name, signature and designation.
- LAI medications dispensed to a named consumer must not be used for another consumer. If this occurs a Datix form must be completed, Pharmacy and Team Leader to be notified.
- Documentation in the Digital Medical Record (DMR) to include a brief , mental state examination, risk assessment and depot administration details as above as per AKG documentation standards – clinical procedure and WA Health Operational Directive OD 052614 State Wide Standardisation Clinical Documentation (SSCD) for Mental Health Service

- In consultation with the respective community team, on discharge from AHS inpatient unit, a prescription for any depot medication will be completed by inpatient medical staff and forwarded to the community team or pharmacist.
- Inpatient unit will send the original depot chart to the clinic via internal mail to avoid duplication and reduce risk of medication errors occurring by having more than one current chart available.

6.2 Refusal to engage with post administration monitoring (Olanzapine)

- Consumers who refuse or lack capacity to consent to be monitored post administration, may not be suitable for ongoing treatment with Olanzapine LAI. In such circumstances, the treating consultant psychiatrist will review regarding continuation or changing to alternative medication.
- Consumers who leave the outpatient clinic prior to the completion of monitoring will have any completed observations reviewed by a medical officer and any actions documented in the health care record. The consumer will be reviewed by their treating team and a plan to achieve compliance or change of medication should be established prior to the next planned administration.
- If a Consultant Psychiatrist considers that the benefit of prescribing Olanzapine LAI to a non-compliant consumer (i.e. those refusing to remain 2 hours at the clinic post administration) outweighs the risks associated with non-compliance of post monitoring, they must document this DMR including the consumers capacity to act accordingly if PIS symptoms occur or whom will be monitoring the consumer outside of the clinic e.g. support worker/family member and that they understand the urgency of seeking medical attention. This must be supported at each visit and will be supported by providing the 'consumer/carer information leaflet (appendix 1). Clinicians administering the LAI will continue to attempt to gain consent to post injection monitoring at each visit.

6.3 Olanzapine Post Injection Syndrome (PIS) monitoring

- PIS occurs when the consumers experience one or more of the signs and symptoms consistent with Olanzapine overdose. In most cases, the signs and symptoms of PIS will be evident within 1 hour of administration.
- Symptoms include:
 - Sedation and/or delirium
 - Confusion, agitation, anxiety
 - Extrapyrarnidal symptoms, dysarthria, ataxia
 - Aggression, dizziness, weakness
 - Hypertension or possible convulsions
- Baseline physiological observations should be taken prior to administration which include blood pressure, temperature, heart rate, respiration rate, O2 saturation levels and levels of consciousness.
- The Adult Observation and Response Chart (AKMR140.3) is to be used to record pre- and post-administration observations, which will be filed in the patients' healthcare record.
- All consumers who receive olanzapine LAI will be monitored for at least 2 hours post injection. This monitoring consists of assessing and documenting at 15-minute intervals for first hour and 30-minute intervals for second hour
 - Orientation
 - Any signs and symptoms of PIS.

- Consumers who develop signs or symptoms of PIS will have a full set of physiological observations completed again and documented and immediate review by their team doctor, available doctor, or duty medical officer (DMO) will be requested.
- For consumers who develop or show signs or symptoms of PIS, the administering clinician is to maintain or increase the frequency of physical observations to 15 min intervals and request an immediate review by the medical team.
- If the consumer meets the Medical Emergency Team (MET) Criteria at any time a 000 call will be placed immediately, and relevant policy followed.
- Consumers who leave the outpatient clinic prior to the completion of the monitoring period, in the outpatient setting, will be contacted by phone by the nurse, team doctor, available doctor or DMO. If the consumer is unavailable for contact by phone, next of kin is to be contacted. If no contact via phone, face to face follow up must be actioned and decision documented accordingly in the health record. This should be discussed with the consultant psychiatrist or treating doctor so that decision to continue olanzapine LAI is reviewed.
- Any adverse effects or instances where the consumer leaves the identified area for monitoring before monitoring is completed will be documented using Datix Clinical Incident Management System.
- Once the monitoring of 2 hours post injection is completed, consumers will be advised to be vigilant for symptoms of post-injection adverse reactions, should be able to obtain medical assistance if needed, and should not drive or operate heavy machinery for 24 hours post administration.

6.4 Clinical Treatment Team (CTT) and Early Episode Psychosis Team (EEPT)

- The RN will liaise with the care coordinator with any issues arising from contact made with the consumer.
- The RN facilitating the clinic will highlight to care coordinators the consumers who have not attended the clinic. The RN will attend the weekly clinical review meetings to highlight remaining overdue depots. The Consultant/Medical Officer will advise an action plan with a named clinician who will action the agreed plan.
- Prior to ordering medications, stock held in the clinic is to be checked against the proposed order to avoid duplication.
- LAI are to be ordered one week in advance of the depot being due via email to AHS Pharmacy from the CTT database available on W. drive. The data base lists all active CTT consumers prescribed LAI medication and is utilised to monitor when depot administration is due, if the consumer is transitioning to GP for administration and for the purpose of ordering medication supply from pharmacy. The CTT depot RN is responsible for maintaining the accuracy of this database prior to forwarding to AHS Pharmacy.
- AHS Pharmacy is to forward the CTT RN a prescription request form for any required prescriptions.
- Care Coordinator / Prescriber are to notify the CTT RN of any discharges or potential discharges, so that the RN can update the depot spreadsheet and forward this information to the pharmacist. This is to ensure depots are not unnecessarily supplied. Depot chart must be ceased by the prescriber. Prescriptions for discharged consumers or active clients receiving LAI prescription from their GP will be archived.
- If depot medication is changed, the care coordinator / prescriber must inform the ESC depot RN immediately either verbally or via email. (so that the depot RN can update the database & pharmacy). The medication change must be reflected on the LAI chart. The depot RN will

also record if the consumer has or has not given consent on the depot clinic database and depot medication chart.

- In the event of a discarded/spoilt depot e.g. a depot is drawn up but consumer refuses administration; expired medication and equipment failure etc. and a replacement being required. The RN will contact the outpatient mental health pharmacist and email AHS Pharmacy dispensary detailing the reasons for a replacement.
- If the depot was not ordered or cannot be located, the clinician is to call AHS Pharmacy to check that it was ordered and organise for one to be collected or delivered to the clinic as soon as possible.
- The pharmacist will carry out monthly checks of the medication held in the storage cupboard at Orchard Avenue Centre (OAC) and return any unused, expired or ceased depots back to AHS Pharmacy.

6.5 Assessment and Treatment Team (ATT) and Older Adult Mental Health (OAMH)

- Procedures for administration of depot medication for these teams should align with the processes listed above. For consumers discharged from the clinic, the prescriber is to update the medication profile and upload the ceased depot chart in the consumers digital medical record.

7. Storage Requirements

- Depot Clinic and Community Clinics – locked/secure room.
- LAI's should not be exposed to temperatures greater than 25°C.
- The LAI injections must be stored in a secure locked cupboard that is kept below 25°C.
- Risperdal® Consta® the entire package is stored in the refrigerator at 2°- 8°C. If no refrigeration is available, it can be stored at temperatures not exceeding 25°C for no more than 7 days prior to administration.

8. Policy context

The following documents are required to give effect to this procedure:

- Legislative
 - Medicines and Poisons Act 2014
 - Mental Health Act 2014
- HDWA
 - [Consent to Treatment Policy](#)
 - [High Risk Medication Policy](#)
 - [Medication Review Policy](#)
 - [State-wide Standardised Clinical Documentation SSCD for Mental Health Services](#)
- EMHS
 - [Patient Consent](#)
 - [Patient Identification and Procedure Matching](#)
- AKG
 - [Medication Policy](#)
 - [Medication Management Procedure](#)

9. Supporting information

The following documents support the implementation of this procedure:

- [Antipsychotic Polypharmacy and High Dose Antipsychotic Prescribing](#)
- [Identification of Patients in the Armadale Mental Health Service](#)
- [Clinical Documentation](#)

10. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated with the documentation of monitoring requirements will be monitored and audited quarterly as per audit standards for the individual mental health service.

11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Clinical Governance
- Standard 4 – Medication Safety - Action 4.3, 4.10, 4.11, 4.12, 4.14, 4.15

12. Glossary

The following definitions are relevant to this policy/procedure.

Term	Definition
Long Acting Injectable Depot	A sustained-action drug formulation that allows slow release and gradual absorption, or that the active agent can act for much longer periods than is possible with standard injections.
Depot Clinic	Outpatient treatment area designed for the administration of an antipsychotic depot injection to eligible consumers by a Registered Nurse (RN).
Consent	A health practitioner performing the procedure (i.e. injection) is responsible for providing information about the specific treatment or procedure and obtaining consent for the medication to be administered
Post Injection Syndrome (PIS)	Signs and symptoms of Olanzapine overdose. One or more of the the signs and symptoms may develop initially within 1 hour of administration. PIS may occur following the administration of Olanzapine LAI despite no prior problematic effects at previous administrations.

13. Policy owner (relating to these procedures)

Service Director Mental Health

Enquiries relating to this procedure may be directed to:

Title: Program Manager – Community Mental Health
 Department: Mental Health
 Email: shannon.stuart@health.wa.gov.au

14. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3.1	26 03 2026	30 07 2028	Minor review, minor changes.

15. Authorisation

This procedure has been authorised and issued by the Service Director Mental Health.

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	26 03 2026
Published date	26 03 2026

Appendix 1

Consumer/Carer Information Sheet After Olanzapine Depot Injection

After this injection, patients are normally observed by clinical staff for 2 hours. This is because, in rare cases, a reaction called Post Injection Syndrome can occur.

If the patient leaves before the 2-hour observation period is complete, it is important that carers know what to watch for and what to do.

What is Post Injection Syndrome?

Very rarely, some of the medication can enter the bloodstream too quickly. If this happens, symptoms can develop within the first few hours after the injection.

Symptoms to Watch For

Seek urgent medical attention if any of the following occur:

Changes in alertness

- Unusual drowsiness
- Confusion
- Disorientation
- Slurred speech

Behavioural changes

- Agitation
- Restlessness
- Unusual irritability

Physical symptoms

- Dizziness
- Weakness
- Unsteadiness or difficulty walking
- Collapse
- Seizure (Convulsion)

If the person becomes difficult to wake, collapses or has a seizure, this is a medical emergency.

What To Do If Symptoms Occur

- Call emergency services on 000.
- Inform paramedics that the person has recently received an olanzapine depot injection.
- **Take the person to the nearest Emergency Department.**

Important Advice For 6 hours After Injection

- They should not drive.
- They should not operate machinery.
- They should not be left alone.
- A responsible adult should stay with them and monitor for symptoms.